## KEY TREATMENTS FOR COPD

- **Medicines** are needed to reduce inflammation and mucus in the airways. In some cases, these medicines could be life-long
- **Regular** respiratory physiotherapy can make the condition manageable and in milder cases can even help a patient resume a normal lifestyle
- **Mild exercises** in consultation with a doctor can improve lung function. These include swimming, brisk walking, cycling, yoga
- **Wear masks** and use air purifiers when air pollution levels deteriorate. Avoid outdoor exercise.
- **Psychological** counselling is recommended to reduce stress
- **Avoid** excessive steam inhalation or homemade remedies without medical consultation

usually so chronic that 10-20 per cent of the lung function gets lost permanently," says Dr Rohatgi.

## LIVING WITH COPD

COPD is usually classified into three stages depending upon the severity of the breathlessness. In the worst case, stage 3, doctors say even sitting down can cause breathlessness. Equally crippling is the constant mucus formation in the chest and throat. "I have to keep spitting out mucus for hours on end and my chest gets so heavily congested that even taking a breath is painful," says Mitra. In some cases, the airways get constricted, leaving no passage for the mucus to pass. COPD also heavily increases the risk of other viral respiratory diseases. "It is a very painful condition and takes a physical and psychological toll on a patient. After all, breath is the essence of life," says Dr Nayar.

COPD patients have no option but to take medication in order to be able to breathe. While steroids are given to only those who are hospitalised,



**"WE ARE SEEING PATIENTS WITH COPD-TYPE OBSTRUCTION IN THEIR AIRWAYS POST COVID, BUT CAN'T SAY IF IT IS PERMANENT"**

**DR SANDEEP NAYAR**
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two main categories of drugs are used for treatment—Lama or long-acting muscarinic antagonists, and Laba or long-acting beta agonists. Lama and Laba are usually taken through inhalers—they help the airways relax, reduce mucal secretions and allow patients to breathe. Unfortunately, though, many patients come at a stage when these drugs alone don't help and have to be paired with antibiotics or steroids. "Most patients come in after infection or severe breathlessness. If they come at the very start of COPD, then Lama and Laba drugs are enough to manage the condition," says Dr Rohatgi.

Along with medication, life with COPD requires regular respiratory rehab. "Few hospitals have such facilities. There is an urgent need for more because the number of patients is increasing," says Dr Sudarshan. Respiratory rehabilitation involves teaching patients exercises that can improve lung capacity. "I recommend swimming, cycling and brisk walking because they can be done at any age without stress on other organs. Exercise, however, comes after a person has received medical treatment," says Dr Rohatgi.

Rehab for COPD patients also involves exercises such as breathing with resistance, or to take out secretions in the throat. "Rehab has a strong role in recovery, and if patients with COPD don't have access to it, then managing the condition can get very hard. With proper rehab, a COPD patient can even start jogging," says Dr Nayar. What doctors do strongly advise against is following breathing videos on the internet. There is no way to find out for certain if the exercises being shown are being done correctly. Doing the first few sessions under supervision helps one to learn how to do the breathing exercises correctly and prevent any further stress to the lungs. "Hospitals are not there just to give medicines," adds Dr Nayar. "At our centre, we have a rehab programme for COPD patients that involves connecting them to a support group of similar patients and to counsellors because it is also a mentally troubling disease." Other logistics such as when to take oxygen, how much to take and even how to safely store oxygen cylinders at home are important to educate COPD patients. "You have to reorganise your life completely once a family member has COPD," says Mishra. "Keeping oxygen tanks at home is very dangerous as they are inflammable."

Psychological counselling for COPD patients is a new part of treatment, one that doctors agree is crucial and relevant. "A patient's muscle mass reduces, there is loneliness as they can't go out all the time. It is also very frightening when you can't breathe," says Dr Rohatgi. Many hospitals ask for COPD patients to come in for respiratory physiotherapy in small groups for encouragement and empathy as they collectively deal with their condition. "Having that circle of people sharing the same challenge is helpful as mental stress can hamper physical recovery," says Dr Singh.

COPD patients and their families have a difficult challenge. Mishra, for instance, says how they all have to wear masks and wash hands before meeting his father. "There is a great risk of passing illnesses to my father," he says. The steps are very similar to Covid—lifelong medication, visits to respiratory rehab, increased psychological stress, the constant risk of hospitalisation, lung infection, breathlessness and lowered immunity. With one difference: COPD is permanent. ■